

Cleft lip and palate

» What is this?

Parts of your baby's face develop independently in the uterus and then grow together. However, if they don't join successfully, a cleft lip and/or palate is the result—these are gaps in the top lip and in the roof of the mouth. A baby may have a cleft lip or a cleft palate, or both. The conditions are often inherited and can hamper your baby's ability to feed successfully, and eventually to talk. They affect more than 7,000 babies in the US each year.

» What can be done?

Your baby will need surgery to bring the two sides of the mouth together, both for the upper lip and the palate. This is usually done by your baby's first birthday; the correction of a cleft lip may be done as early as three months and is followed by a procedure to correct the palate. A few babies may go on to have slight difficulties forming some words or to have a nasal tone to their voice, most recover to have perfectly normal speech. More surgeries may be needed.

Umbilical hernia

» What is this?

This occurs when the baby's intestine pushes out through the hole in her abdominal wall where the umbilical cord was attached, and when this hole fails to close up spontaneously after birth. It is painless, and you may notice the protrusion only when your baby cries or strains. Umbilical hernia is a common condition in newborns, affecting around 3–5 percent of healthy term babies. Premature babies are at slightly increased risk.

» What can be done?

Unless your baby's hernia is large or has become infected, it will probably be left to clear up on its own. Eventually the hole closes and the protruding portion of the intestine goes back into position. Large or infected hernias may need to be treated, possibly with surgery. As long as any infection is treated before it affects the intestine wall, there are no long-term implications.

Inguinal hernia

» What is this?

In around one in six premature babies, part of the small intestine protrudes through a hole in the abdominal wall near the groin, in the space called the inguinal canal. You may notice this hernia only when you see a swelling under the skin near the groin when she cries or strains.

» What can be done?

Your baby will need surgery to push the hernia back into position and close over the hole into the inguinal canal in order to prevent a recurrence. This will usually be done as soon as possible after the hernia has been diagnosed.

Congenital heart problems

» What kinds of defects can occur?

About 40,000 babies born in the US each year (about 1 percent) have a congenital heart defect, so heart problems in newborns are rare. Among the most common is one relating to a special blood vessel—the ductus arteriosus—that enables blood to bypass the baby's lungs while in the uterus. This vessel should close within 24 hours of the birth, but occasionally this doesn't happen, overloading the lungs and heart with blood (see p.311). Other congenital heart problems include:

Atrial septal defect (ASD) and ventricular septal defect (VSD). Commonly referred to as “a hole in the heart,” this is when the two sides of a baby's heart fail to become distinct from one another, causing oxygenated and deoxygenated blood to mix. This is often easily resolved with surgery, and small holes may close up on their own.

Coarctation of the aorta occurs when the aorta (the heart's main artery) is narrowed, limiting blood flow. It can become a serious condition, but is treatable with the insertion of a stent (a small tube that opens the artery), or with surgery to remove the affected part of the aorta.

Stenosis of the valves occurs when the arterial or ventricular valves are narrowed, hampering blood flow between the chambers of the heart and causing a heart murmur. The condition usually requires surgery to correct.

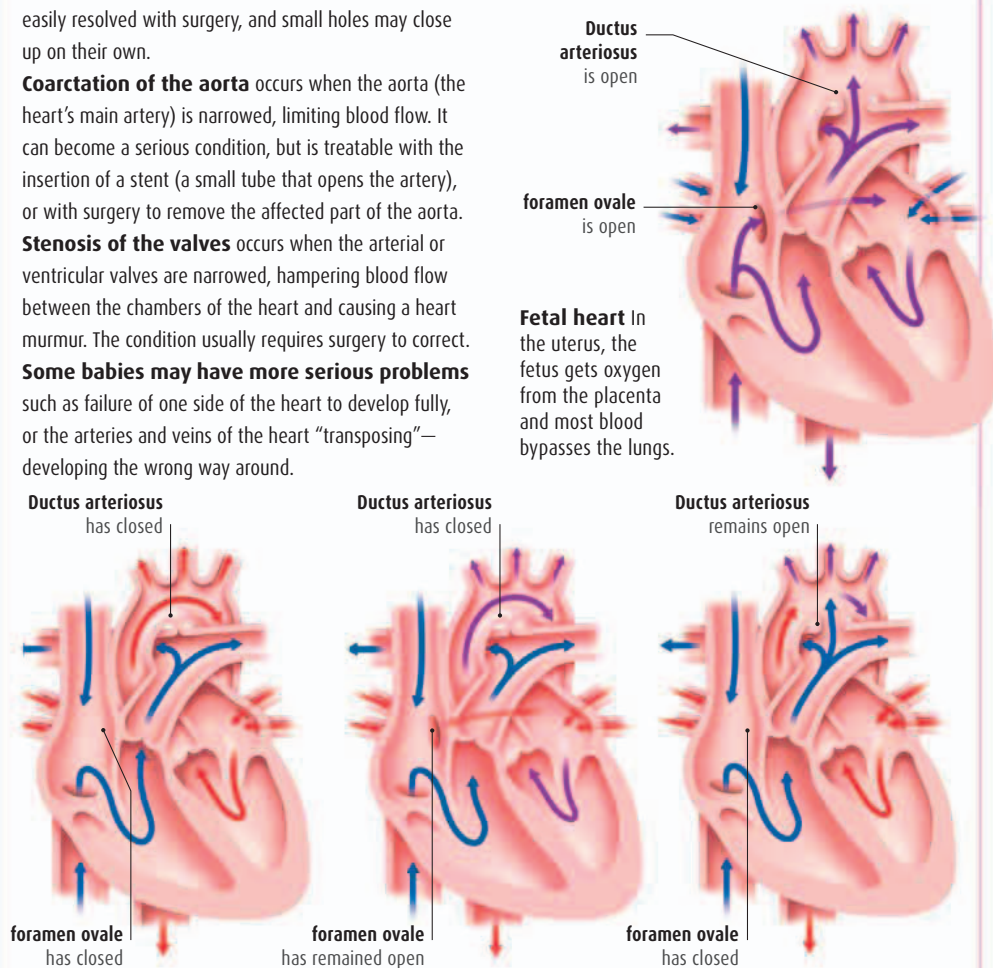
Some babies may have more serious problems such as failure of one side of the heart to develop fully, or the arteries and veins of the heart “transposing”—developing the wrong way around.

» When will heart problems be spotted?

Some heart defects are picked up by the sonographer at your prenatal ultrasounds, others develop at birth. If a problem is spotted, a cardiac specialist will explain the implications of the condition and what treatment, if any, your baby will need when she is born. Ask as many questions as you need to. Your doctor will be able to give you details of support groups, and other parents who have been in a similar situation.

KEY

- Oxygen-rich blood
- Oxygen-poor blood
- Mixed blood



Healthy newborn heart

At birth, a hole (foramen ovale) between the chambers and another at the ductus arteriosus closes—normal circulation begins.

Open foramen ovale

If the hole between the chambers remains open at birth, oxygenated blood returns to the lungs, making circulation less efficient.

Open blood vessel

If the ductus arteriosus fails to shut at birth, oxygenated blood meant for the body is diverted back to the lungs again.